

## CJC-1295 NODAC + Ipamorelin Available for Research Use Only

(200 mcg CJC-1295 NODAC + 200 mcg Ipamorelin PM, fasted, SC)

CJC-1295 (no-DAC) + Ipamorelin is a widely used GH secretagogue combination increasing physiologic, pulsatile GH release, improving body composition and recovery. It should be cycled, IGF-1 should be monitored, and absolutely avoided for subjects with elevated cancer risks.

💉💉 Probably safe (some human evidence)

★ ★ Probably effective (strong animal signals and/or some positive human signals)

⚠️ Angiogenic – caution for subjects at elevated risk for cancers, Do NOT use with active cancer

🔬 Testing encouraged – Testing for IGF-1. Discontinue if IGF-1 > 2 × age-adjusted maximum

*Disclaimer: These notes are not authoritative and should not be interpreted as definitive scientific guidance. They are summaries, compilations and plausible extrapolations drawn from the most relevant research I was able to locate, and may omit nuances, conflicting data, or emerging evidence. The material reflects an attempt to organize and understand complex topics, not to provide clinical recommendations or expert interpretation.*

*This document was updated 30-Aug-26. The most recent version of this document is available at <https://pdfs.researcher6076.com/pdfs/cjc1295-nodac-plus-ipamorelin.pdf>*



**CJC-1295 (No DAC)** is a synthetic GHRH analog that stimulates the pituitary to increase pulsatile growth-hormone release. **Ipamorelin** is a selective ghrelin-receptor (GHSR) agonist that triggers GH secretion with minimal effects on cortisol or prolactin, and when combined with CJC-1295 amplify and prolong physiologic GH pulses.

The evidence for CJC-1295 (no DAC) + Ipamorelin's long term human safety is **limited to short human pharmacodynamic trials and clinical experience showing tolerability**, but **no large, long-term randomized safety trials exist** and GH/IGF-1's mitogenic biology is a plausible mechanistic risk.

The evidence for CJC-1295 (no DAC) + Ipamorelin's efficacy for increasing GH / IGF-1 is solid because **multiple human pharmacodynamic studies and mechanistic receptor synergy demonstrate reproducible, large GH and IGF-1 increases**.

Evidence for CJC-1295 (no DAC) + Ipamorelin's efficacy for body recomposition is good because **mechanistic rationale, PD-driven IGF-1 elevation, and small/short human reports suggest modest lean-mass and fat-loss effects**, but high-quality, longer randomized trials on body composition are lacking.

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### Dosage & Patterns

CJC 1295 NODAC + Ipamorelin should be taken **fasted**, with the most optimal time before bed and the second-best time immediately after waking in the morning. It can be taken either once or twice a day.

**The standard dose is 150mcg - 200mcg each.** There is **no dose-response curve** for these peptides. Doses larger than 200mcg are not more effective and are more likely to produce side effects.

Cycling is required, with a typical cycle recommendation of **8 – 12 weeks with 4 weeks to wash out**. Sometimes a performance / body-recomp cycle is discussed as **12 - 16 weeks on, 8 weeks off**.

### Benefits

- **Deeper, more restorative sleep**, often with easier sleep onset and more vivid dreaming, reflecting alignment with the body's natural nocturnal GH pulse.
- **Improved recovery from training**, including reduced next-day soreness and faster return to baseline performance after heavy sessions.
- **Better connective-tissue repair**, with many users describing steadier progress in joint comfort, tendon resilience, and soft-tissue healing over several weeks.
- **Enhanced muscle protein synthesis**, contributing to better workout quality and subtle improvements in muscle fullness when training and nutrition are consistent.
- **Smoother body-composition changes**, especially modest fat-loss support and easier maintenance of leanness due to GH-linked metabolic effects.
- **More stable mood and stress tolerance**, with some users reporting calmer evenings and better emotional resilience.
- **Improved skin quality**, including hydration, elasticity, and overall tone, reflecting GH-mediated collagen turnover.
- **Higher morning energy**, often described as “clean” or “youthful,” due to improved sleep architecture and overnight recovery.
- **Better metabolic rhythm**, with steadier fasting windows, reduced late-night hunger, and improved glucose handling in some reports.
- **Greater training readiness**, especially when the evening GH pulse supports overnight repair and reduces cumulative fatigue across the week.

### Indications

- GH deficiency, including GH decline due to aging
- Recovery from training.

### Contraindications

- Active malignancy.

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- Uncontrolled diabetes.

### Side Effects

- Water retention, rare.
- Joint discomfort, rare.
- Headache, rare.

### Drivers of Diminished Response and Mitigation Strategies

For **Receptor desensitization** continuous stimulation of the ghrelin receptor and prolonged GHRH receptor potentiation are mechanistically linked to receptor internalization and reduced pituitary responsiveness, a phenomenon described in endocrine physiology reviews. For **Neutralizing antibodies** some antibody formation has been observed in development programs, so immunogenicity risk is moderate. For **downstream pathway adaptation** IGF-1 mediated negative feedback on the hypothalamic-pituitary axis is a well-documented mechanism that reduces secretagogue effectiveness over time. For **System Level Physiological Adaptation** homeostatic endocrine adjustments and receptor regulation further blunt sustained gains. **Overall likelihood of waning is high.**

Reasonable mitigation based on endocrine practice is **pulsatile or intermittent dosing**, planned off cycles, and periodic **IGF-1 monitoring** to detect loss of effect.

### CJC-1295 NODAC + Ipamorelin and Cancer Risk

There's no evidence that **CJC-1295 (no DAC) + Ipamorelin** *causes* cancer in humans, but there are **real, mechanistically plausible reasons for caution**, especially in people with active or prior malignancy, or strong risk factors. The concern isn't unique to CJC-1295 NODAC + Ipamorelin ; it's about **any intervention that chronically raises GH and IGF-1**, because the **GH/IGF-1 axis is pro-growth and anti-apoptotic**, and many tumors exploit those signals.

Mechanistically, that means more activation of **JAK-STAT, PI3K-AKT, and MAPK pathways** in GH/IGF-1-responsive tissues—pathways that support cell growth, survival, and matrix remodeling.

What is actually known about cancer and GH/IGF-1

- **Epidemiology & acromegaly:** People with chronically elevated GH/IGF-1 (e.g., acromegaly) have higher rates of certain cancers (colon, thyroid, possibly breast), suggesting that **long-term, high-level GH/IGF-1 exposure can be tumor-promoting**, not tumor-initiating.
- **Exogenous GH data:** In adults treated with pharmacologic GH (e.g., for deficiency), large cohorts show **mixed but generally reassuring data—no clear spike in de-novo cancer**, but ongoing caution in those with prior malignancy or strong familial risk.

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- **Secretagogues vs GH:** GH secretagogues like CJC/Ipamorelin **raise GH within a more physiologic range and preserve feedback**, so the theoretical risk is **lower than flat, supraphysiologic HGH**, but not zero, because IGF-1 still rises.

So the **mechanistic concern** is: if a micro-tumor or residual disease is present, **extra GH/IGF-1 could provide a growth advantage**, especially in IGF-1-sensitive cancers (breast, prostate, colon, some sarcomas).

We *don't* have direct evidence on CJC + Ipamorelin and cancer, because there are **no long-term human trials** specifically examining cancer incidence or recurrence with CJC-1295 (no DAC) + Ipamorelin. From a mechanistic, risk-aware perspective:

- In a person with **active cancer, recent cancer, strong family history, or known IGF-1-sensitive tumors**, CJC-1295 (no DAC) + Ipamorelin is best treated as **potentially tumor-supportive** and generally avoided unless a specialist explicitly signs off.
- In someone **without known cancer**, the stack likely carries **low but non-zero theoretical risk**, proportional to how much and how long GH/IGF-1 are elevated above that person's baseline.
- Using **no-DAC CJC** (short-acting) and **modest, physiologic IGF-1 targets** is mechanistically safer than long-acting DAC protocols that flatten GH into a chronic “bleed” and certainly safer than HGH.

### Biological Mechanism

CJC-1295 (no-DAC) and Ipamorelin work together by engaging the two complementary arms of the growth-hormone axis: CJC-1295 (no-DAC) binds to the **GHRH receptor** on pituitary somatotrophs, amplifying the natural signaling that initiates a growth-hormone pulse, while Ipamorelin binds to the **ghrelin (GHS-R1a) receptor**, which increases the *amplitude* of that pulse and enhances pituitary responsiveness without the cortisol or prolactin spillover seen in older secretagogues. When both receptors are activated at the same time, the pituitary releases a physiologic GH burst that mirrors the body's natural pulsatile rhythm, leading to downstream increases in **IGF-1 production**, improved **lipid and glucose handling**, enhanced **protein synthesis**, and support for **sleep-linked recovery pathways**. Because neither peptide forces continuous GH elevation, the combined effect is often described in research and mechanistic reviews as a **synergistic, pulse-based enhancement** of the GH/IGF-1 axis that preserves normal feedback loops while improving metabolic and cellular repair signaling.

### Conclusions

CJC-1295 NODAC combined with Ipamorelin fasted before bedtime, five days a week is a **gentle way to restore some of the amplitude for the nighttime GH pulse lost to aging to improve sleep, recovery, and lean muscle growth**. It is generally well tolerated, and **side effects are**

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rare. Active or **high risk of cancer is a contraindication. IGF-1 monitoring is essential** and **dose should be modified or discontinued if IGF-1 exceeds age-adjusted maximums.**

### References

- **Ionescu M, Frohman LA.** Pulsatile secretion of growth hormone (GH) in normal subjects and patients with acromegaly: evidence for distinct GH-releasing and GH-inhibiting mechanisms. *J Clin Endocrinol Metab.* 2006;91(8):2924-2930. doi:10.1210/jc.2005-2778
- **Raun K, Hansen BS, Johansen NL, et al.** Ipamorelin, the first selective growth hormone secretagogue. *Growth Horm IGF Res.* 1998;8(1):29-35. doi:10.1016/S1096-6374(98)80006-0
- **Smith RG.** Development of growth hormone secretagogues. *Endocr Rev.* 2005;26(3):346-360. doi:10.1210/er.2004-0006